

Risk Management Dashboard — Story Guide

Dashboard: Risk Management | **Pages:** Overview, Risk Capture Potential | **Last updated:** [Date]

About this guide

This guide explains the story the Risk Management dashboard tells — from the headline view of risk position down to individual HCCs and targetable patient cohorts for outreach. The dashboard spans two pages: Overview identifies and diagnoses coding gaps; Risk Capture Potential converts those gaps into a prioritized outreach strategy.

Risk adjustment is the process of accurately documenting the clinical complexity of a population through HCCs (Hierarchical Condition Categories), which determines payments from CMS, HHS, or state Medicaid agencies. Higher and more accurate risk scores reflect the true cost of caring for a population. Effective risk management means closing coding gaps — discrepancies where conditions exist clinically but aren't fully documented in claims — so reimbursement aligns with the actual clinical burden.

The funnel:

- **Top** → What is the risk position? (KPI cards, population and recapture summary)
 - **Middle** → Where are the coding gaps? (Payer breakdowns, trends, risk models, attribution, clinical risk factors)
 - **Bottom** → Who is responsible and what's driving it? (Practice/PCP detail, disease-level recapture, gap closure patterns)
 - **Action** → Who do we target and how? (Risk Capture Potential page — outreach segmentation)
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Page 1: Overview

Layer 1: The risk position

KPI cards — risk summary

 *Insert: Screenshot of both rows of KPI cards — Eligible Population, Documented Risk, Potential Risk, Gap to Potential Risk (row 1) and PMPM, Risk Recapture Rate, RAF Recapture Rate, % Members with Open Coding Gaps (row 2).*

The KPI cards at the top represent the overall risk position. They are arranged in two rows — the first row describes the risk landscape, the second row measures performance against it.

Row 1: The risk landscape

Eligible population

The total number of individuals eligible for risk adjustment in the selected period. The Members sub-count shows the attributed members on whom risk has been executed.

Direction	Interpretation
Increasing	Growing population — more members to manage. Check whether risk infrastructure is scaling accordingly.
Decreasing	Shrinking population — risk metrics may improve mechanically without reflecting actual coding improvements.

Documented risk

The current documented risk score, calculated based on the standard risk models specific to the LOB (CMS HCC for Medicare Advantage, HHS HCC for Commercial, CDPS for Medicaid). Risk scores are calculated based on various HCCs triggered for each member, along with their demographics. This is the score the organization will be paid on.

Direction	Interpretation
Increasing	More HCCs being captured from claims and supplemental encounters — reflects accurate clinical documentation.
Decreasing	Either the population is genuinely healthier, or HCC capture is slipping. Cross-check with Potential Risk to distinguish.

Potential risk

The maximum achievable risk score if all open coding gaps are closed. Composed of documented risk plus dropped HCCs (Recapture codes — chronic conditions identified in past years but not yet in the current period) and suspected HCCs (Suspect codes — clinically indicated by lab tests, medications, vitals, or procedures but never formally coded). Represents the true clinical burden of the population.

Direction	Interpretation
Increasing	Population's clinical burden is growing — more conditions are present or being identified by Recapture and Suspect algorithms.
Decreasing	Population getting healthier, or members with high HCC burden disenrolling.

Documented Risk should always be read alongside Potential Risk. The distance between them is the uncaptured opportunity that Recapture and Suspect codes are designed to close.

Gap to potential risk

The difference between Potential Risk and Documented Risk — the risk score associated with all open coding gaps. Composed of Recapture (chronic HCCs from prior years not yet recaptured) and Suspect (HCCs indicated by clinical evidence but never formally coded). The sub-text shows the split: e.g., "Dropped + Suspected (0.168 + 0.038)".

Direction	Interpretation
Decreasing	Recapture and Suspect codes are being addressed — coding gaps are closing.
Increasing	Gap is widening — chronic conditions are dropping off year-over-year, or new suspected HCCs are being identified faster than they're being coded.
Stable while documented risk grows	Potential Risk is also growing — clinical burden of the population is shifting upward.
Recapture component much larger than Suspect	Most gaps are previously documented chronic conditions — typically easier to close because clinical evidence already exists.
Suspect component growing	Clinical evidence (labs, meds, vitals) is identifying new HCCs faster than they're being coded — needs physician review and validation.

Row 2: Performance metrics

PMPM

Per Member Per Month cost for the population. Provides financial context — populations with higher documented risk should have correspondingly higher PMPM. A mismatch between PMPM and documented risk often signals undercoding.

Direction	Interpretation
Decreasing	Lower per-member cost — could reflect efficiency or could indicate underfunding if documented risk is not keeping pace.
Increasing	Higher per-member cost — verify that risk scores are tracking proportionally. High PMPM with low documented risk indicates revenue at risk.

Risk recapture rate

The percentage of HCC risk factors identified in the current year that were also documented in the previous year. Specifically focuses on chronic conditions that should persist year-over-year. A high rate reflects strong coding habits and accurate carryover of documented diagnoses.

Direction	Interpretation
Increasing	More chronic HCCs from prior years are being recaptured in the current period — strong documentation continuity.
Decreasing	Chronic conditions are being missed in the current period despite being documented previously — annual documentation workflow is breaking down.

RAF recapture rate

The percentage of risk score (RAF value) that has been recaptured. Unlike Risk Recapture Rate which counts HCCs equally, RAF Recapture Rate weights each HCC by its financial impact — some HCCs carry more RAF value than others.

Direction	Interpretation
Increasing	Financially impactful HCCs are being recaptured — protects reimbursement.
Decreasing	High-value HCCs are dropping off — reimbursement at risk even if total HCC counts look acceptable.

Risk Recapture Rate and RAF Recapture Rate should be read together. If Risk Recapture is high but RAF Recapture is low, low-weight HCCs are being captured while high-weight ones are being missed — coding effort is misallocated. If RAF Recapture is higher than Risk Recapture, high-value HCCs are being prioritized appropriately.

% Members with open coding gaps

The percentage of attributed members who have at least one open coding gap (Recapture or Suspect HCC not yet closed). Measures the breadth of the gap problem across the population.

Direction	Interpretation
Increasing	More members have unaddressed gaps — Recapture and Suspect identification is outpacing closure.
Decreasing	Fewer members with gaps — gaps are being closed through encounters, or gap-heavy members are disenrolling.

Reading the cards together

Pattern	Interpretation
Gap to Potential ↓, RAF Recapture ↑, Risk Recapture ↑	Recapture and Suspect codes are being effectively closed — accurate risk documentation is improving.
Gap to Potential ↑, both recapture rates ↓	Coding gaps are widening across the population — annual chronic condition documentation is failing.
RAF Recapture ↑ but Risk Recapture ↓	High-value HCCs being prioritized but lower-value ones are being missed — strategically efficient but creates compliance exposure.
Risk Recapture ↑ but RAF Recapture ↓	Many HCCs being captured but the financially impactful ones are slipping — coding focus needs realignment.
% Open gaps ↑ while recapture rates are stable	New suspected conditions are being identified by clinical algorithms faster than gaps are being closed through encounters.
PMPM declining while Documented Risk is also declining	Risk scores not keeping pace with cost trajectory — potential underfunding for the actual clinical burden.

Population and risk distribution — LOB chart and payer table


Insert: Screenshot of the Across LOBs bar chart and Payer/plan details table.

The Across LOBs chart and Payer/plan table decompose the headline KPIs by line of business and payer.

Across LOBs

Compares RAF recapture rate across Commercial, Medicaid, and Medicare Advantage. The previous year vs current year comparison shows whether each LOB is improving or declining.

Pattern	Interpretation
One LOB significantly lower than others	That LOB has a structural recapture problem — may stem from payer-specific data feeds, provider coding patterns under that LOB's risk model, or population-specific characteristics.
All LOBs declining similarly	System-wide documentation issue — not LOB-specific. Look at practice-level patterns and coding workflows.
Medicare Advantage performing differently from Commercial	Expected to some degree — MA risk adjustment programs are typically more mature and have stronger documentation infrastructure. The gap between LOBs indicates opportunity.

Payer/plan details

Breaks down every KPI by payer and plan. Key columns:

Column	What to look for
Documented risk	Payers with low documented risk relative to PMPM may be undercoded.
Gap to potential risk	Payers with the largest gaps have the most coding opportunity — prioritize for chart review and provider engagement.
RAF recapture rate	Primary performance metric per payer. Compare with YoY change to assess trajectory.
RAF RR YoY change	Negative values mean recapture is declining for that payer — investigate data feed completeness, provider attribution changes, or population shifts.
Risk recapture rate	HCC-level capture per payer. Compare with RAF recapture to assess whether coding effort is aligned with revenue impact.
PMPM	Financial context — high PMPM with low documented risk suggests undercoding for that payer's population.

A payer with high PMPM but low documented risk has the largest revenue exposure — care costs are high but risk-adjusted payments aren't reflecting the true burden. These payers should be prioritized for chart review, suspect HCC validation, and provider documentation support.

Layer 2: The diagnosis

Trends over time

 *Insert: Screenshot of all six trend lines — Members, Eligible Population, Documented Risk vs Potential Risk, Risk Recapture Rate, RAF Recapture Rate, PMPM.*

The trend lines add the time dimension. Each chart compares the current year trajectory against the previous year, month by month.

Members and Eligible Population trends

Membership trajectory provides the denominator context. If membership is growing, flat recapture rates mean recapture is scaling proportionally. If recapture rates decline while membership grows, coding workflow isn't keeping up.

Documented risk vs Potential risk trend

The most important trend on the dashboard. It plots three lines — previous year documented, current year documented, and potential risk. The visual gap between current documented and potential is the uncaptured opportunity.

Pattern	Interpretation
Current documented rising toward potential	Recapture and Suspect codes are being closed — coding gaps narrowing over time.
Current documented flat while potential rises	Gap widening — clinical burden growing but documentation isn't keeping pace.
Current documented running parallel to potential	Gap stable — coding maintaining current performance but not improving.
Seasonal dip in documented risk	Normal early in the measurement year — documented risk accumulates as encounters occur and chronic HCCs get recaptured.

Risk Recapture Rate and RAF Recapture Rate trends

Track recapture performance month over month. Both rates typically start low in January and build through the year as encounters accumulate and chronic HCCs get re-documented.

A recapture rate that plateaus mid-year suggests the easily recaptured HCCs have already been closed — remaining gaps require targeted intervention through wellness visits, pre-visit planning, and Suspect code validation. This is when the Risk Capture Potential page becomes most valuable.

PMPM trend

Monthly cost per member. Spikes in specific months may indicate high-cost episodes or seasonal utilization patterns. The relationship between PMPM trend and documented risk trend reveals

whether the financial trajectory aligns with risk capture — if PMPM grows but documented risk doesn't, undercoding is likely.

Risk models and sub-models

 *Insert: Screenshot of the Risk model/sub-models table.*

The Risk model/sub-models table breaks down performance by the specific risk adjustment model applied to each population segment. Risk scores are calculated using standard models defined for each LOB — CMS HCC for Medicare Advantage, HHS HCC for Commercial, and CDPS for Medicaid. Each model has sub-categories (Adult, Child, Community, Disabled, TANF, etc.) shown as expandable child rows.

This table matters because different populations are scored under different methodologies, and recapture performance varies across them. Key columns:

Column	What to look for
Eligible population	Size of each model's population — large populations have more impact on overall RAF metrics.
Gap to potential risk	Which model has the largest uncaptured opportunity.
RAF recapture rate	RAF performance per model — a model at 96% needs less intervention than one at 75%.
Risk recapture rate	HCC-level capture per model.
Open gaps (Dropped)	Recapture targets — chronic HCCs documented in prior years but not yet in the current period.
Open gaps (Suspected)	Suspect targets — HCCs indicated by clinical evidence (labs, meds, vitals, procedures) but never formally coded.

Recapture (Dropped) gaps are typically easier to close than Suspect gaps because clinical evidence already exists from prior documentation — the condition was previously coded. Suspect gaps require new clinical assessment by the physician to validate the diagnosis before coding.

Pattern	Interpretation
One model has significantly lower recapture than others	That model's population may have data feed gaps, different provider coding patterns, or HCC types that are harder to recapture under that methodology.
High Recapture (Dropped) gaps with moderate recapture rate	Significant opportunity remains — chronic HCCs were documented before, evidence exists for re-coding.
Low Suspect gaps relative to Recapture (Dropped)	Most gaps are recapture opportunities, not new documentation — operationally easier to close through wellness visits.
Sub-model performance varies widely within a parent model	Some sub-populations (e.g., Adult vs Child within HHS) are managed differently — investigate workflow differences.

Risk breakdown by attribution status

 *Insert: Screenshot of the Risk breakdown by attribution status table.*

This table segments the population by attribution status — Continued (members present in both prior and current periods), Discontinued (members who have left), and Newly Enrolled — and shows risk metrics for each. Reveals whether coding gaps are concentrated in the stable population, departing members, or new entrants.

Status	Expected behavior	Red flag
Continued	Should have the highest recapture rates — these members have historical HCC documentation that should carry forward annually.	Low recapture here means chronic conditions documented in prior years aren't being re-documented in the current period — workflow problem, not a data problem.
Discontinued	Gap to potential risk may remain high — coding closure for departing members is operationally lower priority.	High open gaps here represent revenue lost to disenrollment before HCCs were captured for the current year.
Newly Enrolled	Low documented risk is expected — limited encounter history in this system.	High potential risk with low documented risk means the new members' clinical conditions exist but haven't been documented through encounters yet — first-year documentation priority.

Newly enrolled members with high Gap to Potential Risk represent the most time-sensitive opportunity — their HCCs are real (identified by Suspect algorithms from claims and clinical data) but haven't been formally coded in this organization. Wellness visits and PCP attribution within the first months of enrollment are critical.

Practice/PCP details

📷 Insert: Screenshot of the Practice/PCP details table and PCP distribution scatter plot.

The Practice/PCP table translates the risk story into specific provider names. It shows which practices are capturing HCCs effectively and which are leaving documented risk on the table.

Key columns:

Column	What to look for
Gap to potential risk	Practices with the largest gaps have the most uncaptured HCCs — direct documentation improvement targets.
RAF recapture rate	Primary performance metric per practice.
RAF RR YoY change	Trajectory — is this practice improving documentation or declining?
% members with open coding gaps	Breadth — what proportion of this practice's panel still has unaddressed Recapture or Suspect codes.
Open gaps (Dropped + Suspected)	Absolute volume — a practice with 20% open gaps and 10,000 members has more total gaps than one with 30% on 500 members.

Reading patterns:

Pattern	Interpretation
High RAF recapture + low gap to potential	Practice is capturing HCCs effectively — model performer.
Low RAF recapture + high gap to potential	Significant uncaptured HCCs — direct candidate for InNote engagement, chart review, or coding education.
High % open gaps + high member count	Largest absolute volume of unclosed gaps — highest-impact practice for outreach support.
Very high RAF recapture (98%+) + very low gap to potential	Near the documentation ceiling — incremental investment in this practice has limited upside.
Large negative RAF RR YoY change	Practice's documentation performance is declining — needs intervention even if absolute rate looks acceptable.

PCP distribution scatter

Plots individual PCPs by member volume (x-axis) against Gap to Potential Risk (y-axis, configurable). PCPs in the upper-right (high volume, high gap) are the highest-impact targets — they have many members with significant uncaptured HCCs, so a single intervention with this provider moves multiple gaps.

Risk factor details and recapture by disease


Insert: Screenshot of the Risk recapture rate by disease bar chart and Risk factor details table.

The Risk factor details table and disease-level recapture chart move from "which practice" to "which HCCs" — identifying the specific clinical risk factors where recapture is succeeding or failing.

Risk recapture rate by disease

Horizontal bar chart showing recapture rate for each disease cohort (Diabetes, COPD, CKD, Vascular Disease, etc.). Conditions at the far left (low recapture) are the HCCs being missed most frequently year-over-year.

Pattern	Interpretation
A condition has very low recapture (<40%)	That HCC category is being systematically missed — needs targeted coding education, chart review focus, or InNote prompts for that disease cohort.
A condition has high recapture (>80%)	Documentation is well-embedded for this HCC category — workflow is functioning.
Chronic conditions (Diabetes, COPD, CKD, CHF) with low recapture	Highest-priority concern — these are persistent conditions that must be re-documented annually for accurate risk scores. Low recapture means annual documentation isn't happening.

Risk factor details table

Column	What to look for
Risk factor / HCC code	The specific HCC category (e.g., HCC37 for Diabetes with Chronic Complications, HCC280 for COPD).
Eligible population	How many members have this HCC — larger populations have more revenue impact.
Gap to potential risk	The risk score value at stake for this HCC across the population.
Risk recapture rate	What percentage of this HCC has been recaptured.
Risk RR YoY change	Trajectory — is recapture for this HCC improving or declining?
Open gaps (Dropped)	Recapture targets — this HCC was documented in prior years but not yet in the current period. Highest priority because clinical evidence exists.
Open gaps (Suspected)	Suspect targets — clinical evidence (labs, meds, vitals, procedures) suggests this HCC but it has never been formally coded.

HCCs with large eligible populations, high gap to potential, and low recapture rates represent the highest-value documentation targets. Prioritizing these moves the overall RAF recapture rate most efficiently because the HCC affects many members and carries significant revenue weight.

Gap closure patterns

 *Insert: Screenshot of all three gap closure donut charts — by type of visit, by network status, by provider type.*

The three gap closure donuts show how gaps are actually being closed — through what visit types, in what network setting, and by what provider type. These charts aren't diagnostic; they're operational intelligence about where HCC documentation is happening.

Gap closure by type of visit

Segment	Interpretation
Outpatient dominant	HCCs are being documented in routine outpatient visits — the most scalable and cost-effective channel.
Inpatient contributing significantly	Some HCCs are only being documented during hospitalizations — these conditions are being missed in outpatient settings where they should be captured proactively.
Professional visits contributing	Specialist or ancillary encounters are closing gaps — may indicate HCCs that require specialty assessment to document.

Gap closure by network status

Segment	Interpretation
Mostly in-network	HCCs being documented within the contracted network — controllable through provider engagement and InNote workflow.
Significant out-of-network	HCCs being documented at OON facilities — less controllable. May indicate network gaps for specialties needed to document specific HCCs.

Gap closure by provider type

Segment	Interpretation
PCP-driven	HCC documentation embedded in primary care workflow — sustainable and scalable through annual visits and chronic care management.
Specialist-driven	Certain HCCs only being captured by specialists — may need to expand PCP coding capability so chronic conditions are documented annually in primary care.
"Others" dominant	Non-traditional encounters closing gaps — investigate what "Others" includes (telehealth, home visits, etc.) and whether it's a reliable, repeatable channel.

Page 2: Risk Capture Potential

The Risk Capture Potential page shifts from "what is the gap" to "who do we target." It takes the open coding gaps identified on the Overview page and segments the targetable patient population by outreach-relevant dimensions, enabling the creation of recommended cohorts (chase lists) for maximum impact.

 *Insert: Screenshot of the Risk Capture Potential page header showing Across LOBs bar chart and Payer/plan details table.*

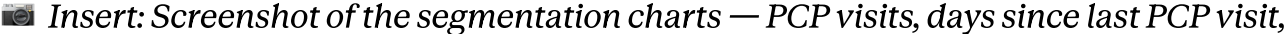
Targeted population by LOB and payer

The **Across LOBs** bar chart shows the volume of targeted patients by line of business. The **Payer/plan details table** shows, for each payer:

Column	What to look for
Latest risk execution	Date of the most recent risk model run for this payer/plan.
Targeted patients	Members with open coding gaps eligible for outreach.
Targeted gaps	Total open Recapture and Suspect codes across those patients.
Risk recapture rate (current)	Where HCC capture stands today.
Potential risk recapture rate	The ceiling if all targeted gaps were closed (always 100%).
RAF recapture rate (current)	Where RAF capture stands today.
Potential RAF recapture rate	The ceiling for RAF capture — the difference from current rate is the revenue upside.

The gap between current and Potential RAF Recapture Rate is the documented revenue opportunity per payer. Payers with the largest gap should be prioritized for chase list generation, InNote engagement, and provider outreach.

Outreach segmentation

 *Insert: Screenshot of the segmentation charts — PCP visits, days since last PCP visit,*

wellness visit status, cost of care, risk bucket, care gaps bucket, coding gaps bucket.

The segmentation charts distribute targeted gaps across dimensions that determine outreach strategy.

Across PCP visits (rolling 12 months)

Segment	Interpretation
Patients with 0 PCP visits	Not engaging with primary care — hardest to reach. Health plan-level outreach (mail, phone, member services) needed.
Patients with 1-4 visits	Engaged but gaps still open — encounters happened but coding didn't capture the chronic HCCs. Documentation workflow issue, not access issue.
Patients with 5+ visits	Frequently seen, gaps still open — persistent documentation miss with the same provider. Likely needs chart review or InNote prompts for that PCP.

Across days since last PCP visit

Segment	Interpretation
31-60 days	Recently seen — high-priority for pre-visit planning at next appointment. Retrospective chart review can also catch missed HCCs from the last visit.
61-180 days	Moderate gap — proactive scheduling outreach to bring patient back for AWW or chronic care visit.
181-365 days	Drifting from care — needs targeted outreach campaign to re-engage before the measurement year ends.
365+ days	Lost to follow-up — hardest to re-engage. Health plan intervention or transitions of care opportunities.

Across wellness visit status

Segment	Interpretation
Completed but gaps remain	AWV happened but didn't capture all chronic HCCs — documentation improvement opportunity for the provider who conducted the visit. May need retrospective chart review.
Due	AWV not yet completed — scheduling this single visit is the highest-value outreach action because AWWs are designed to capture all chronic HCCs comprehensively.

Members with "Due" wellness visits and high open gap counts are the ideal cohort targets — one comprehensive AWV can close multiple Recapture and Suspect codes simultaneously.

Across cost of care (rolling 12 months)

Segment	Interpretation
<\$1K with open gaps	Low utilizers — chronic conditions may be undermanaged as well as undocumented. Care management and documentation opportunity together.
\$50K+ with open gaps	High utilizers whose HCCs still aren't fully documented — significant revenue exposure and care management priority.

Across risk bucket

The dashboard categorizes members into risk buckets based on documented risk score, with thresholds varying by risk model:

Risk Bucket	CMS	HHS-HCC	CDPS
Low	0 - 0.99	0 - 0.49	0 - 0.39
Medium	1 - 1.99	0.50 - 3.99	0.40 - 0.99
Rising risk	2 - 3.99	4 - 7.49	1 - 1.99
Very High	4+	7.5+	2+

Segment	Interpretation
Low risk with targeted gaps	HCCs that would elevate the member's risk bucket once documented — ensures resource allocation matches actual clinical complexity.
Very High risk with targeted gaps	Already high-risk members still with uncaptured HCCs — both revenue impact and care management urgency are highest here.

Across care gaps bucket / Across coding gaps bucket

Distribution of gap volume per member. Members with concentrated gap volume (9+ gaps) represent high-efficiency outreach targets — a single comprehensive encounter can close many gaps at once.

Geographic and access dimensions

 *Insert: Screenshot of the zip map, PCP-member distance chart, and ED visit chart.*

Across zip (top 10)

Map showing geographic concentration of targeted patients. Clusters indicate areas where a focused outreach campaign, mobile clinic, or community health event would reach the most members.

Across PCP - member distance

Segment	Interpretation
5-10 miles	Reasonable access — barriers are unlikely geographic. Focus outreach on scheduling and engagement.
20+ miles	Access barrier — telehealth visits or mobile health units may be more effective than in-office encounters for closing HCCs.

Across ED visit bucket (last 6 months)

Segment	Interpretation
0 ED visits	Not using ED — patient is engaging differently or not at all with the healthcare system.
1+ ED visits with open gaps	Patient is in the system via emergency care but chronic HCCs aren't being captured during those encounters. ED visits rarely close coding gaps — these members specifically need a PCP encounter.

Practice/PCP targeting

 *Insert: Screenshot of the Across Practice/PCP table on the Risk Capture Potential page.*

The Practice/PCP table on this page ranks practices by total targeted gap volume — the absolute number of open gaps concentrated in each practice's panel. This is the operational prioritization list for provider outreach, InNote engagement, and chart review investment.

Pattern	Interpretation
A practice has high targeted gaps but performed well on Overview	New gaps may have emerged from population changes (new attributions) or newly identified Suspect codes — not necessarily a documentation failure.
A practice has high targeted gaps and was low-performing on Overview	Consistent underperformance — needs structured intervention through coding education, workflow changes, or chart review support.
Top 3-5 practices account for majority of total gaps	Concentrated opportunity — addressing a few practices moves the overall recapture metrics significantly.

How the funnel connects

Layer	Section	Question it answers
Top	KPI cards (Row 1)	What is the documented vs potential risk position?
Top	KPI cards (Row 2)	How well are HCCs being captured at the condition and revenue level?
Top	LOB chart + Payer table	How does recapture vary by payer and line of business?
Mid	Trend lines	When did gaps emerge, and is the trajectory improving?
Mid	Risk model table	Which risk model and population segment has the largest uncaptured HCC opportunity?
Mid	Attribution status table	Are continued, discontinued, or newly enrolled members driving the gap?
Bottom	Practice/PCP table	Which providers are documenting HCCs effectively and which are not?
Bottom	Risk factor details	Which specific HCCs are being missed across the population?
Bottom	Gap closure donuts	Through what channels — visit type, network, provider type — are HCCs actually being documented?
Action	Risk Capture Potential — payer table	Which payers have the most targetable revenue opportunity?
Action	Segmentation charts	How should outreach be structured — by visit recency, AWW status, access, risk tier, gap volume?

Layer	Section	Question it answers
Action	Geographic/access charts	Where are the patients, and what barriers exist?
Action	Practice targeting	Which practices should receive InNote engagement, chart review, or coding education first?

Each layer answers the question the previous one raises. Overview identifies and diagnoses uncaptured HCCs. Risk Capture Potential converts that diagnosis into a targeted chase list and outreach strategy.

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